

VOREN®

Voren, the brand of Diclofenac Sodium, is an anti-inflammatory, analgesic and antipyretic agent. Chemically it is Sodium a - [o - (2, 6 dichloroanilino) phenyl] acetate.

Presentation(s):

Voren Enteric Coated Tablet 25mg:

Each tablet contains:

Diclofenac Sodium 25mg

Voren Enteric Coated Tablet 50mg:

Each tablet contains:

Diclofenac Sodium 50mg

Pharmacology (Summary of Pharmacodynamic and Pharmacokinetics):

Pharmacodynamics:

1. Voren is a non-steroidal-inflammatory agent that possesses analgesic and antipyretic activities. Its mode of action, like that of other non-steroidal anti-inflammatory agent, is not known; however, its therapeutic action is not due to pituitary-adrenal stimulation, but may be from the inhibitory action in the synthesis of prostaglandin.
2. Voren exhibits marked antirheumatic, anti-inflammatory, analgesic and antipyretic properties and is thus suitable for use in inflammatory and degenerative rheumatic diseases, as well as for the treatment of post-traumatic inflammation and swelling.
3. The tablets are covered with an acid resistant protective coating, with the result that the active substance is not released until it has passed through the stomach.

Pharmacokinetics:

1. Diclofenac Sodium is a phenylacetic acid derivative which has analgesic, antipyretic and anti-inflammatory actions. It is used mainly in the treatment of rheumatoid arthritis and other rheumatic disorders.
2. Diclofenac Sodium is absorbed from the gastrointestinal tract, peak plasma concentrations occur about 2 hours after ingestion of enteric coated tablets. At therapeutic concentrations it is more than 99% bound to plasma protein. It is metabolized and excreted mainly in the urine. Small amount are excreted in the bile. The amount excreted in the urine account for 20 to 30% of the dose and that in bile for 10 to 20%. Then mean terminal elimination half life of Diclofenac Sodium is 1.2 to 1.8 hours and metabolite possesses about 1/30 of the anti-inflammatory activity of parent drug.

Indication(s):

Inflammatory and degenerative forms of rheumatism; rheumatoid arthritis; ankylosing spondylitis and osteoarthritis.

Non-articular rheumatism, post-traumatic inflammation and swelling.

Dosage and Administration:

Osteoarthritis:

100 ~ 150mg/day in divided doses. Dosage above 150mg/day has not been studied in patients with osteoarthritis.

Rheumatoid arthritis:

150 ~ 200mg/day in divided doses. Dosage above 200mg/day has not been studied in patients with rheumatoid arthritis.

Ankylosing spondylitis:

100 ~ 125mg/day in divided doses. Dosage above 125mg/day has not been studied in patients with ankylosing spondylitis.

As a general recommendation, the dose should be individually adjusted. Adverse effects may be minimized by using the lowest effective dose for the shortest duration necessary to control symptoms (see section Precaution/Warning).

Established cardiovascular disease or significant cardiovascular risk factors:

Treatment with diclofenac is generally not recommended in patients with established cardiovascular disease (congestive heart failure, established ischemic heart disease, peripheral arterial disease) or uncontrolled hypertension. If needed, patients with established cardiovascular disease, uncontrolled hypertension, or significant risk factors for cardiovascular disease (e.g. hypertension, hyperlipidaemia, diabetes mellitus and smoking) should be treated with diclofenac only after careful consideration and only at doses ≤100mg daily if treated for more than 4 weeks (see section Precaution/ Warning).

Mode of Administration:

To be taken orally.

Contraindication:

Contraindicated with patients with severe cardiac failure (see section Precaution/ Warning).

Side Effect(s) / Adverse Reaction(s):

Gastrointestinal side effects are the most frequent adverse effects of Diclofenac Sodium. Other rare side effects includes insomnia, irritability agitation, headache, dizziness, vertigo, pruritus, depression, drowsiness, minor hearing disorders edema, skin rashes and palpitations, blurred vision, liver function disorder (jaundice). These side effects however, are usually mild in nature and generally disappear after a few days despite continuation of the medication.

Dermatological: rashes or skin eruptions, cases of hair loss, bullous eruptions, erythema multiforme, Steven's-Johnson Syndrome, toxic epidermal necrolysis (Lyell's syndrome) and photosensitivity reactions have been reported.

Cardiac disorders:

Uncommon: Myocardial infarction, cardiac failure, palpitations and chest pain. The frequency reflects data from long-term treatment with a high dose (150mg/day).

Not known: Kounis syndrome

Description of selected adverse drug reactions

Arteriothrombotic events

Meta-analysis and pharmacoepidemiological data point towards an increased risk of arteriothrombotic events (for example myocardial infarction) associated with the use of diclofenac, particularly at a high dose (150mg daily) and during long-term treatment (see section Precaution/Warning).

Precaution(s) / Warning(s):

1. Hypertension. NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking antihypertensive with NSAIDs may have an impaired antihypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAID treatment and at regular intervals thereafter.
2. Heart Failure. Fluid retention and oedema have been observed in some patients taking NSAIDs, hence caution is advised in patients with fluid retention of heart failure.
3. Gastrointestinal Events. All NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation which may increase with dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g. the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulcerations occur in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patient about signs and symptoms of serious gastrointestinal toxicity. The concurrent use of aspirin and NSAIDs also increases the risk of serious gastrointestinal adverse events. NSAIDs, including diclofenac, may be associated with increased risk of gastrointestinal anastomotic leak. Close medical surveillance and caution are recommended when using diclofenac after gastrointestinal surgery.
4. Severe Skin Reactions. NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reactions and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.
5. Close monitoring is needed when used in patient with asthma and bronchospasm.
6. Cardiovascular effects. Treatment with NSAIDs including diclofenac, particularly at high dose and in long term, maybe associated with an increased risk of serious cardiovascular thrombotic events (including myocardial infarction and stroke). Treatment with diclofenac is generally not recommended in patients with established cardiovascular diseases (congestive heart failure, established ischemic heart diseases, peripheral arterial diseases) or uncontrolled hypertension. If needed, patients with established cardiovascular diseases, uncontrolled hypertension, or significant risk factors for cardiovascular disease (e.g. hypertension, hyperlipidaemia, diabetes mellitus and smoking) should be treated with diclofenac only after careful consideration and only at doses ≤ 100 mg daily when treatment continues for more than 4 weeks.

As the cardiovascular risks of diclofenac may increase with dose and duration of exposure, the lowest effective daily dose should be used for the shortest duration possible. The patient's need for symptomatic relief and response to therapy should be re-evaluated periodically, especially when treatment continues for more than 4 weeks.

Patients should remain alert for the signs and symptoms of serious arteriothrombotic events (e.g. chest pain, shortness of breath, weakness, slurring of speech), which can occur without warnings.

Patients should be instructed to see a physician immediately in case of such an event.

7. Severe cutaneous reactions, including Stevens - Johnson syndrome and toxic epidermal necrolysis (Lyell's syndrome), have been reported with diclofenac sodium. Patients treated with diclofenac sodium should be closely monitored for signs of hypersensitivity reactions. Discontinue diclofenac sodium immediately if rash occurs.

WARNINGS

RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAID

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (eg. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious adverse events and other risk factors associated with peptic ulcer disease (eg. alcoholism, smoking, and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

Pregnancy and Lactation:

Use in pregnant women has not been established. Nor is Diclofenac Sodium recommended for use in nursing mothers because it will be excreted in the milk.

Symptoms and Treatment for Overdosage, and Antidote(s):

There is no experience with acute overdosage. Should accidental overdosage occur, the stomach should be emptied by inducing emesis or by careful gastric lavage followed by the administration of activated charcoal. Vital functions should be monitored and supported. Because it is firmly bound to plasma proteins, hemodialysis and peritoneal dialysis may be of little value.

Shelf-Life:

4 years from the date of manufacture.

Storage Condition(s):

Keep in a tight container. Store at temperature below 30°C. Protect from light and moisture.

Packing(s):

Plastic bottle of 1000's & 1500's (for export only).

Blister packing of 10's x 10 and 10's x 100.

Product Description:

Voren Enteric Coated Tablet 25mg: A yellow color enteric coated round tablet.

Voren Enteric Coated Tablet 50mg: An orange yellow color enteric coated round tablet.



Manufacturer and Product Registration Holder:
Y.S.P. INDUSTRIES (M) SDN. BHD. (199001001034)
Lot 3, 5, 7 & 12, Jalan P/7, Section 13,
Kawasan Perindustrian Bandar Baru Bangi,
43000 Kajang, Selangor, Malaysia.
Ordering Line: 1 800 88 3027
Product Info: 1 800 88 3679